جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : كامله حسن عبد المجيد مراد
المحافظة : القاهره **الحي / المدينة :** السيده زينب **العنوان :** مممممممممم
رقم البطاقة : 26103030100907 **رقم التليفون :** 01117026720 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 03-03-1961 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
الإخصائي الاجتماعي
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبطات الاروماتيز [14161] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: كامله حسن عبد المجيد مراد
National ID: 26103030100907

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



سونار وماصوجرام




جمهورية مصر العربية
بطاقة تحقيق الشخصية
كامله
حسن عبدالمجيد مراد
٣ ح عليفى على حسن حسنى
السيدة زينب - القاهره
٢٦١٠٣٠١٠٠٩٠٧
١٩٦١/٠٣/٠٣
KE0603971


٢٦١٠٣٠١٠٠٩٠٧
٢٠٢٣/٠٩
ربة منزل
أنثى
مسلمة
١
البطاقة سارية حتى ٢٠٣٠/٠٩/١٥

Name: كامله حسن عبدالمجيد مراد	
MR. : 5426	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 65-year-old individual who is postmenopausal. The clinical history notes a prior right-sided breast cancer treated with breast-conserving surgery on 11/7/2021 (T2 N1), followed by adjuvant chemotherapy (3 cycles of AC) and subsequent loss to follow-up until August 2025, when a local recurrence in the right breast with metastasis to the left axilla was identified. Both lesions were biopsied, revealing estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, and HER2 amplification by SISH (CT4B). The patient subsequently received 19 weeks of taxane-based chemotherapy and six cycles of trastuzumab (Herceptin). The case was presented for multidisciplinary discussion regarding surgery versus continued hormonal therapy and trastuzumab.

The pathology confirms a recurrent invasive carcinoma with ER 8/8, PR 8/8, HER2 SISH amplified, and a Ki-67 index of 4 % (CT4B). Imaging to date includes a mammosonography that will be reviewed, and a bone scan is pending to complete the metastatic workup. No additional metastatic sites have been documented beyond the left axillary involvement.

The referring team requested multidisciplinary input on the options of surgery versus continued hormonal therapy and trastuzumab. The MDT concluded that, given the oligometastatic recurrence, the results of the mammosonography should be obtained and a bone scan performed, after which the case will be reconvened for a surgical decision. In the interim, the patient will remain on trastuzumab and an aromatase inhibitor.

تحريرا في ,

التاريخ :	23-09-2026	الوقت :	

الطبيب المعالج :

يعتمد ,

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient's Name:	kamala hassan abdalmaged morad
Date:	2026-04-05
Referred by:	Prof.Dr. ct cap iv
Hospital:	Goustave Roussey International Egypt

MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

TECHNIQUE: Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

FINDINGS:

Clinically reported right breast carcinoma.

(I) Chest:

- Most of both breast is out of field of examination
- Few enlarged left axillary lymph odes , the largest measures 18 x 11 mm.
- Mosaic attenuation of both lung lobes .
- Clear both lung fields .no sizable pulmonary masses or consolidation patches
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

(II) Abdomen and pelvis:

- The liver is enlarged in size showing homogenous CT density , no focal lesions
- Patent and homogenously enhancing portal vein.
- Calcular gall bladder harboring multiple hyperdense stones .

GUSTAVE ROUSSY
INTERNATIONAL EGYPT

Patient's Name:	kamala hassan abdalmaged morad
Date:	2026-04-05
Referred by:	Prof.Dr. ct cap iv
Hospital:	Goustave Roussey International Egypt

- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions.
- No vertebral structural collapse .

OPINION:

- No pulmonary , hepatic or bony metastatic deposits.

Dr.Reham Khedr, MD

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	كاملة حسن عبدالمجيد	Age	62
Sex	female	Special habits	
Family History		Comorbidity	Hypertensive
Surgical History		Telephone	01117026720
Menopausal Status	Postmenopausal	Marital Status	
ID		Medical ID	2196/2021
Date	8/10/2025	Reason of MDT	

Medical Report

Pathology: IDC

ER: 8/8. PR: 8/8. HER-2: 2+. Ki67: 40%

cT4b N0 M1

S.M:

Metsworkup:

Recommendations :

- Post menopausal recurrent Rt breast cancer with mets to the contralateral axilla (left axillary LNs)
- Previous history of rt BCS in 11/7/2021 (T2 N1)
- Recieved 3 cycles of adjuvant AC and FU till 7/2025
- Newly developed rt breast retroareolar mass + left axillary LN

MDT DECISION	For: TO START IV HERCEPTIN + TAXOL With regular clinical examination Consider palliative local RTH if there is response

Name	د/ ندي حسن	د/ مني زكي	
Signature			

USTAVE OUSSY

Patient Name	: الأستاذه / كاملة حسن عبد المجيد	Request No.	: 13003669
Patient No.	: 26103030100907	Request Date	: 21/08/2025
Path No.	: 2604/25	Verification Date	: 30/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 64 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Amplified with small clusters .

Signature
Dr. Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey .

Verified by :
Dr.Heba Hasan

CAP ACCREDITED

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY

Patient Name	: الأستاذه / كاملة حسن عبد المجيد	Request No.	: 13003669
Patient No.	: 26103030100907	Request Date	: 21/08/2025
Path No.	: 2604/25	Verification Date	: 30/08/2025
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 64 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data
Right breast mass with left enlarged axillary lymph node enlargement .

Nature of specimen
Tru cut biopsy, collected on 20/8/2025.

Gross examination
1) Right breast :
Multiple grayish white tissue cores up to 1 cm, totally submitted.
2) Nodal tissue :
Positive for metastatic deposits .
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
1)Right breast mass :
* Histological type: Invasive lobular carcinoma .
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* DCIS: Absent
* Angioinvasion: Absent.
*Perineural invasion: Absent.
*Microcalcification : Absent .
• IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
* HER2: Moderate complete membrane reaction in>10 % of tumor cells (score 2+)

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Kamla Hassan Abdel Maguid Mourad	Date : 11/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 64 Year(s)
Medical ID: 2196/2021	Pat. ID: 94137

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 64-year-old female with past history of right CBS and ALAND, a month ago had pathologically proven right ILC and metastatic right axillary lymph node level1.

Findings:

Right breast.

- Preserved fibroglandular pattern.
- Thickened skin reaching 11 mm, and coarsened trabeculae.
- Multiple scattered at all quadrants irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study; largest seen retro-areolar measuring 38 x 32 mm invading the nipple. Another lesion seen 12 o'clock, invading the overlying skin measuring 31 x 20 mm. (progressive course)
- Clear right axilla. No pathological infra or supraclavicular lymph nodes.

Left breast.

- Preserved fibroglandular pattern.
- no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- one pathological axillary LN (level 1) is noted showing cortical thickness reaching 11 mm measuring 26 x 11 mm. No pathological infra or supraclavicular lymph nodes

Conclusion:-

Pathologically proven metastatic left axillary level 1 node.

Right breast (BIRADS 6): Multi-centric locally advanced malignant process.

Left breast (BIRADS 1):Normal study.

CESM is recommended to exclude occult left breast cancer.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Patient Name: Kamla Hassan Abdel Maguid Mourad	Date : 16/09/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 64 Year(s)
Medical ID: 2196/2021	Pat. ID: 94137

CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH IV CONTRAST: -

No available prior imaging studies for comparison:

Findings:

Chest:

- Few bilateral tiny pulmonary nodules with average size 3 mm.
- Bilateral diffuse mosaic attenuation of both lungs , otherwise normal both lung parenchyma.
- No mediastinal lymph nodes enlargement.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal appearance of the thoracic aorta and the great vessels.
- Right breast showed suspicious retroareolar speculated lesion averaging 3.3 cm with related small satellites ...for Sono mammographic assessment.
- *Enlarged left axillary lymph node about 23 mm for US assessment.*

Abdomen and pelvis:

- Average size liver showing homogenous density. No gross focal lesions
- Average size spleen, no focal lesions.
- Calcuar gall bladder., No dilated intra or extra-hepatic bile ducts.
- Urinary bladder shows no masses or stones.
- Normal C.T. appearance of the pancreas, adrenal glands, both kidneys, bowel loops & other pelvic organs.
- No retroperitoneal lymphadenopathy.
- No ascites.
- **The examined bones show no worrisome lytic or sclerotic lesions.**

Opinion:

- ***Bilateral few tiny pulmonary nodules...Nonspecific for follow up.***
- ***Right breast showed suspicious retro-areolar speculated lesions with related small satellites ...for Sono mammographic assessment.***

Dr. Reda Mabrouk, MD

Patient's Name:	**Kamila Hassan Abdul Majid Mourad**
Date:	**2026-07-26**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: folloe up after right CBS recurrence underwent NACT , follow ups study.

Findings:

Right breast.

- Predominant fatty breast tissue . (ACR A)
- Thickened right breast skin with underluing large central infilterative mass ,
- **Ultrasound multiple central malignant masses are seen mthe largest is seen retroareolar in location measuring (4 x 3 cm)**
- **One pathological level 1 right axillary LN measuring (1.6 x 1.2 cm)**

Left breast.

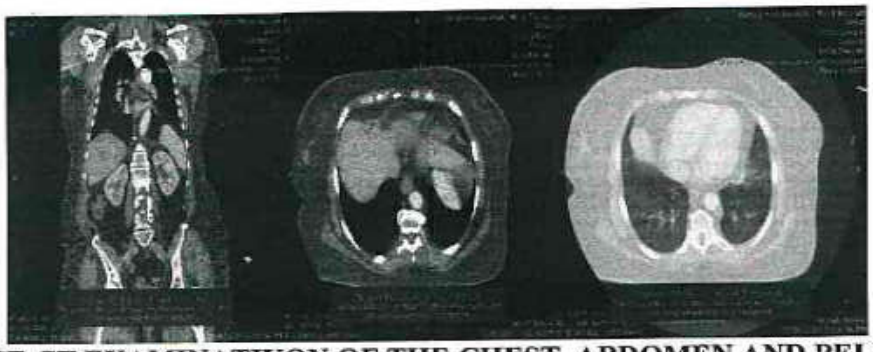
- Predominant fatty breast tissue . (ACR A)
- No detected masses ,distortion or pathological calcification .
- **Ultrasound** demonstrated no suspicious solid or cystic masses .
- Normal skin/nipple/areola complex.
- **The previously described level 1 left axillary LN measures (1.8 x 1 cm).**

Conclusion:-

- ***Right breast (BIRADS 6): Stable disease .***
- ***Left breast (BIRADS 6): Regressive course as regard the previously described LN.***

Prof.MONA ZAKY

Patient's Name:	**Kamila Hassan Abdul Majid Mourad**
Date:	**2026-07-28**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**



MULTISLICE CT EXAMINATIUON OF THE CHEST ,ABDOMEN AND PELVIS WITH CONTRAST ADMINISTRATION REVEALED:-

- Evidences of surgical intervention in the right breast,with an enhancing mass lesion still seen at 9 O'clock position of the breast and measures about 21x16 mm in dimensions. The overlying skin is still thickened,more in the peri-areolar region and measures about 6.2 mm.
- **Normal left breast.**
- **Benign-looking left axillary lymph nodes are seen with preserved their hilar fat contents and the largest one showed thickened its cortex and measures about 18x13 mm in dimensions,likely to be a hyperplasia.**
- Hyperinflated both lungs.
- Bilateral basal bronchiectatic changes with wall thickening and peribronchial pneumonitis
- No sizable abnormally-enhancing pulmonary nodules ,other patchy areas of consolidations or foci of calcifications noted.
- No sizable abnormally-enhancing mediastinal or hilar lymph nodes seen.
- Patent central airways.
- No pleural or pericardial effusions.
- Average size of the heart with normal CT of its big vessels.
- **Normal bones of the thoracic cage apart from spondylo-degenerative changes of the scanned spine with degeneration scattering in discs,more at multiple lumber levels.**
- Increased size of the liver, measuring about 17.1cm ,showing diffuse reduced parenchymal attenuation and a homogenous enhancement pattern.
- No sizable abnormally-enhancing focal lesions or dilated biliary radicles.
- Patent PV.
- Multiple variable sized stones are seen in the gall bladder ,globally measuring about 37x23mm with its CT density of 515 HUs.
- Average sized spleen,measuring about 11.3 cm. No focal lesions seen inside.
- Normal CT appearance of the pancreas, functioning both kidneys, both suprarenal glands, aorta and IVC.
- No sizable abdominal or pelvic lymphadenopathy.
- No ascites.

Patient's Name:	**Kamila Hassan Abdul Majid Mourad**
Date:	**2026-07-28**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- No intra-vesical stones,solid masses,wall thickening or diverticulosis seen.
- The rest of the pelvic structures are unremarkable.

IMPRESSIONS:

- **Evidences of surgical intervention in the right breast,with an enhancing mass lesion still seen at 9 O'clock position of the breast. The overlying skin is still thickened,more in the peri-areolar region.**
- **Hyperinflated both lungs with bilateral basal bronchiectatic changes with wall thickening and peribronchial pneumonitis.**
- **Fatty infiltrated liver.**
- **Calcular cholecystitis.**
- **For clinical check up.**

Prof/Hanan El-Ahwal.

Patient Name: Kamla Hassan Abdel Maguid Mourad	Date : 11/09/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 64 Year(s)
Medical ID: 2196/2021	Pat. ID: 94137

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: 64-year-old female with past history of right CBS and ALAND, a month ago had pathologically proven right ILC and metastatic right axillary lymph node level1.

Findings:

Right breast:

- Preserved fibroglandular pattern.
- Thickened skin reaching 11 mm, and coarsened trabeculae.
- Multiple scattered at all quadrants irregular spiculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and mild internal vascularity on color Doppler study; largest seen retro-areolar measuring 38 x 32 mm invading the nipple. Another lesion seen 12 o'clock, invading the overlying skin measuring 31 x 20 mm. (progressive course)
- Clear right axilla. No pathological infra or supraclavicular lymph nodes.

Left Breast:

- Preserved fibroglandular pattern.
- No solid or cystic lesions .
- Normal skin/nipple/areola complex.
- One pathological axillary LN (level 1) is noted showing cortical thickness reaching 11 mm measuring 26 x 11 mm. No pathological infra or supraclavicular lymph nodes.

Conclusion:

Pathologically proven metastatic left axillary level 1 node.

Right breast (BIRADS 5): Multi-centric locally advanced malignant process.

Left breast (BIRADS 1):Normal study.

CESM is recommended to exclude occult left breast cancer.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspition.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name: Kamla Hassan Abdel Maguid Mourad	Date : 28/07/2025
Exam : MAMMOGRAPHY BOTH	Age : 64 Year(s)
Medical ID: 2196/2021	Pat. ID: 94137

FOLLOW UP BILATERAL BREAST MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
Right BCS, with palpable lump

TECHNIQUE:
- Bilateral MLO and CC views
- Bilateral breast ultrasound

REPORT:
- Fairly dense fibro glandular density, (ACR A).

RIGHT BREAST:
- The multiple ill defined spiculated masses are still seen, yet exhibiting significant increase in size, with the largest located in the retroareolar region.
- Complementary U/S revealed underlying heterogenous hypoechoic solid masses seen mainly in the outer quadrants and to a lesser extent the inner quadrants, with the largest located retroareolarly measuring about 3x3.5cm, and overlying dermal invasion.
- Extensive vascularity is detected within on color Doppler application.
- No pathological axillary lymph node is are seen and free infra and supra clavicular regions.

LEFT BREAST:
- No dominant mass, spiculation or suspicious micro calcification, this was further confirmed by an U/S examination.
- A pathological level I axillary node is seen, measuring 27x28mm, with free infra and supra clavicular regions.

OPINION:
- Post right BCS with progressive course of the locally advanced neoplastic process, (BIRADS 6), with a left level I pathological axillary node.

Best regards,
Dr. Mahi Mokhtar
Signature Dr.Mahi Mokhtar
28/07/2025 11:24 AM